Hope Hospital		
Creation Date 01/01/2017		**Guidelines on Verbal Orders and Approved List of Medications**
Released Date 11/01/2017	Version: 3.0	
Reviewed On :- 16/01/2023 Reviewed On :- 15/01/2025	**Created by Quality Coordinator** Signature/Name: Dr. Darpan Jakkal,	**Guidance and Approved by Director** Signature/Name: Dr. B K Murali

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1. Purpose

To ensure safe, accurate, and timely communication of verbal medication orders and to minimize medication errors in compliance with NABH standards.

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2. Scope

These guidelines apply to all healthcare professionals involved in medication prescribing, administration, and documentation in the hospital.

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3. Definition of Verbal Order

A **verbal order** is a medication order communicated verbally (face-to-face or via telephone) by a licensed prescriber to a registered healthcare professional, usually in emergency or urgent situations.

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4. Conditions for Verbal Orders

- Verbal orders should be **exceptional** and only used when the prescriber is not able to provide a written or electronic order promptly (e.g., emergency).
- Should be limited to situations where delay in medication administration could adversely affect patient care.

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5. Approved List of Medications for Verbal Orders (per NABH guidance)

Verbal orders are generally allowed only for:

- **Emergency medications** (e.g., adrenaline, atropine)
- **Critical care medications** (e.g., inotropes, sedatives)
- **Life-saving drugs** (e.g., thrombolytics)
- **Immediate antibiotic therapy** when delay may worsen infection
- **Insulin and anticoagulants** (with caution and proper verification)

Note: The hospital's Pharmacy and Therapeutics Committee (PTC) should prepare and regularly update the list based on clinical needs and safety.

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6. Process and Documentation

- **Verification:** The receiver of the verbal order (nurse or pharmacist) must repeat the order back to the prescriber to confirm accuracy.
- **Documentation:** The verbal order must be documented immediately in the patient's medical record, including:
    - Name of the medication
    - Dose, route, frequency
    - Date and time of the order
    - Name and designation of the prescriber

    - Name and designation of the person receiving the order

- **Prescriber's Signature:** The prescriber must sign and date the verbal order within 24 hours or as per hospital policy.

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7. Responsibilities

- **Prescriber:** Give clear, unambiguous verbal orders only when necessary; confirm in writing ASAP.
- **Receiver (Nurse/Pharmacist):** Confirm and document the order accurately; clarify any doubts immediately.
- **Pharmacy:** Monitor verbal orders and alert for any inconsistencies or unusual orders.

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8. Training and Audit

- Regular staff training on the importance and procedures for verbal orders.
- Periodic audits to ensure compliance and identify potential errors.

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9. Precautions

- Avoid verbal orders for high-risk medications unless absolutely necessary.
- Use standardized communication tools like SBAR (Situation, Background, Assessment, Recommendation) during verbal orders.